Hemostatic powders as monotherapy or a rescue therapy in malignancy-related gastrointestinal bleeding: urgent need for large randomized controlled trial

To the Editor:

We read with interest the article by Karna et al[1] on the efficacy of topical hemostatic agents, mainly hemostatic powder (HP), in malignancy-related GI bleeding (GIB). This condition is an endoscopic challenge, and a criterion standard hemostatic technique remains to be defined. The metanalysis, including 16 studies with 530 patients, revealed a primary hemostasis rate of 94.1%, with an aggregate (early and delayed) rebleeding rate of 24.2%. A limitation of the study lies in the scarcity of published data, as stated by the authors themselves. Despite an extensive evaluation of >350 citations, the authors could select only 2 randomized controlled trials—a pilot including 10 patients[2] and a preliminary abstract on 18 patients[3]—and 14 studies with different methodologies and heterogeneous lesions. The disease stage affects the outcome; it is hard to achieve stable hemostasis in diffuse lesions with multiple bleeding points. HP is a straightforward procedure, well suited for malignancy-related GIB because of the possibility of spreading the hemostatic agent over a large area of bleeding. A key point is whether it is preferable to use HP as a first choice or rescue therapy, namely, as an alternative to, or after the failure of, other hemostatic procedures. In our experience, HP used as a first choice or rescue therapy achieved immediate hemostasis in 100% and 85.7% but definitive hemostasis (no rebleeding) in 45.5% and 85.7%, respectively, of patients with malignancy-related GIB.[4] Taken together, the available data preclude drawing a definitive conclusion as to whether HP is an alternative to other hemostatic methods in malignancy-related GIB. Prospective large multicenter studies are urgently needed to define the appropriate use of HP, as a standalone or rescue therapy, and whether it may be considered the criterion standard in malignancy-related GIB.

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REFERENCES

1. Karna R, Deliwala S, Ramgopal B, et al. Efficacy of topical hemostatic agents in malignancy-related gastrointestinal bleeding: a systematic review and meta-analysis. Gastrointest Endosc. Gastrointest Endosc 2023;97:202-8.
2. Chen Y, Wyse J, Lu Y, et al. TC-325 hemostatic powder versus current standard of care in managing malignant GI bleeding: a pilot randomized clinical trial. Gastrointest Endosc 2020;91:321-8.e1.
3. Da Costa Martins B, Scomparin RC, Bento LH, et al. Preliminary results of a randomized controlled trial comparing hemostatic powder versus optimal clinical treatment in the management of gastrointestinal bleeding from malignancy [abstract]. Gastrointest Endosc 2018;87:AB415-A416.
4. Paoluzi OA, Cardamone C, Aucello A, et al. Efficacy of hemostatic powders as monotherapy or rescue therapy in gastrointestinal bleeding related to neoplastic or non-neoplastic lesions. Scand J Gastroenterol 2021;56:1506-13.

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Response:

We thank Paoluzi et al[1] for their encouraging letter in response to our recently published meta-analysis on the efficacy of topical hemostatic agents in malignancy-related GI bleeding (GIB).[2] Despite our literature search efforts, only 2 randomized controlled trials were available for inclusion, and we were unable to answer the key question of whether topical Hemospray (Cook Medical, Bloomington, Ind, USA) could be a preferable first-choice therapy or rescue therapy. We agree with Paoluzi et al[1] that prospective large multicenter studies are needed to define the utility of hemostatic agents as a first-line or rescue agent in comparison with other hemostatic methods.

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REFERENCES

1. Paoluzi OA, Sibilia M, Monteleone G, et al. Hemostatic powders as monotherapy or a rescue therapy in malignancy-related gastrointestinal bleeding: urgent need for large randomized controlled trial. Gastrointest Endosc 2023;97:378.
2. Karna R, Deliwala S, Ramgopal B, et al. Efficacy of topical hemostatic agents in malignancy-related gastrointestinal bleeding: a systematic review and meta-analysis. Gastrointest Endosc. Gastrointest Endosc 2023;97:202-8.

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Hybrid argon plasma coagulation after endoscopic mucosal resection—some caveats in the comparison with snare tip soft coagulation

We read with great interest the study by Motchum et al,[1] who analyzed the performance of hybrid argon-plasma coagulation (APC), a technique whereby APC is preceded by submucosal injection of saline through a high-pressure water jet[2] after EMR of large colonic lesions. The reduction of post-EMR recurrence rates through the use of APC or snare-tip soft coagulation (STSC) on the normal-appearing scar margins is well documented.[3,4] Prior studies had also suggested that STSC may be preferable to APC.[5]

In this study, Motchum and et al[1] found lower recurrence in the first surveillance colonoscopy after hybrid APC (2.2%) than that reported for STSC (6.0%, in a recent analysis), as well as a lower percentage of serious side effects (3.6% versus 8.5%[6]).[6] These results are excellent and allow hypothesizing that hybrid APC may be the method of choice, potentially because of the safer ablation of deeper tissue.[7] Notwithstanding, some caveats arise, including the small sample size, the single-arm design, and the lack of standardization of the hybrid APC technique, given that most patients (72.0%) underwent ablation of both scar surface and margins, hampering comparison with prior STSC studies. No recurrence at 6 months was observed in another recent pilot study[8] of hybrid APC for colonic large lesions where scar surface ablation was performed in all patients, being plausible that such ablation may justify the lower recurrence rate. Another drawback is the lack of cost effectiveness analysis. In fact, although STSC does not involve additional costs, hybrid APC carries an incremental cost effectiveness ratio of $158.7/1.0% (or $603.0 per procedure for a 3.8% decrease in recurrence), and fixed costs of around $50,000 for Erbejet2 and APC3 modules (prices for Portugal). Additional studies are needed to compare head-to-head standardized STSC, APC, and hybrid APC, to evaluate characteristics of the lesions that may lead to selecting 1 technique over another, and to evaluate cost effectiveness.

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REFERENCES

1. Motchum L, Levenick JM, Djinbachian R, et al. Endoscopic mucosal resection combined with hybrid argon plasma coagulation to prevent recurrence of large nonpedunculated colorectal polyps. Gastrointest Endosc 2022;96:840-8.e2.
2. Kolb JM, Shah S, Chahine A, et al. Hybrid argon plasma coagulation for Barrett's esophagus. VideoGIE 2021;6:339-41.
3. Dhillon AS, Ravindran S, Thomas-Gibson S. Recurrence after endoscopic mucosal resection: there's more to it than meets the eye. Gastrointest Endosc 2021;94:376-8.
4. Klein A, Tate DJ, Jayasekeran V, et al. Thermal ablation of mucosal defect margins reduces adenoma recurrence after colonic endoscopic mucosal resection. Gastroenterology 2019;156:604-13.e3.
5. Meulen LWT, Bogie R, Winkens B, et al. Thermal ablation of mucosal defect margins to prevent local recurrence after endoscopic mucosal resection of large nonpedunculated colorectal polyps: a systematic review and meta-analysis. Gastrointest Endosc 2022;95:AB187-8.
6. Chandan S, Facciorusso A, Ramai D, et al. Snare tip soft coagulation (STSC) after endoscopic mucosal resection (EMR) of large (>20 mm) no pedunculated colorectal polyps: a systematic review and meta-analysis. Endosc Int Open 2022;10:E74-81.
7. Maselli R, Belletrutti PJ, Spadaccini M, et al. Fluid cushion protects against thermal damage during argon plasma coagulation. Ann Gastroenterol 2021;34:845-51.
8. Motz VL, Lester C, Moyer MT, et al. Hybrid argon plasma coagulation-assisted endoscopic mucosal resection for large sessile colon polyps to reduce local recurrence: a prospective pilot study. Endoscopy 2022;54:580-4.

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Are sex-specific benchmarks for clinically significant serrated polyp detection warranted?

To the Editor:

We read the recently published article by Anderson et al[1] and the accompanying editorial by Macaron et al[2] with interest. The study demonstrated that higher detection rates of clinically significant serrated polyps (CSSP-DRs) are associated with lower rates of postcolonoscopy colorectal cancer. The findings support using CSSP-DR as an independent quality measure for colonoscopy. Although target adenoma detection rates are largely agreed upon, there remains heterogeneity of serrated polyp definitions in various